

Demographics

Patient Name: Wei Chen

DOB: 09/14/1975 (Age: 50)

Sex: Male

MRN: 9912034

Encounter Info

Date of Service: 05/05/2026

Encounter Type: Emergency Department (ED) – Initial Evaluation

Provider: Dr. Sarah Lin, MD (Emergency Medicine)

Chief Complaint

Acute abdominal pain.

History of Present Illness (HPI)

The patient is a 50-year-old male presenting to the Emergency Department with complaints of severe abdominal pain that began approximately 12 hours ago. He describes the pain as starting periumbilically last night but has since localized to the right lower quadrant (RLQ). The pain is sharp, constant, and exacerbated by movement, currently rated as a 7/10 in severity. He reports associated mild nausea and anorexia but denies any episodes of emesis. He denies fever, chills, diarrhea, constipation, hematochezia, dysuria, or gross hematuria. No prior history of similar abdominal pain or abdominal surgeries.

Review of Systems (ROS)

- Constitutional: Negative for fever or chills.
- Gastrointestinal: Positive for abdominal pain, nausea, and anorexia. Negative for vomiting or diarrhea.

- Genitourinary: Negative for dysuria or flank pain.
- Cardiovascular: Negative for chest pain or palpitations.
- Respiratory: Negative for shortness of breath.

Vital Signs

- BP: 120/80 (Taken: 08:30 AM, Right Arm, Seated)
- HR: 76 bpm
- RR: 16 breaths/min
- Temp: 98.6 °F (Oral)
- SpO2: 99% on room air
- Wt: 175 lbs (79.4 kg)

Physical Examination

- General: Patient appears mildly uncomfortable, guarding his abdomen, but in no acute cardiopulmonary distress.
- HEENT: Normocephalic, atraumatic. Mucous membranes are moist.
- Cardiovascular: Regular rate and rhythm. Normal S1, S2. Excellent distal perfusion.
- Pulmonary: Lungs clear to auscultation bilaterally.
- Abdominal: Bowel sounds are normoactive. Abdomen is soft but demonstrates significant tenderness to palpation localized to the right lower quadrant. Positive McBurney's point tenderness. Negative Murphy's sign. No obvious rebound tenderness or rigidity, though voluntary guarding is present.
- Skin: Warm, dry, intact. No rashes or jaundice.

Laboratories / Diagnostics

- CBC with Differential: WBC $11.5 \times 10^3/\mu\text{L}$ (mild leukocytosis with a left shift). Hgb 14.2 g/dL. Platelets $250 \times 10^3/\mu\text{L}$.
- CMP: Sodium 140 mEq/L, Potassium 4.0 mEq/L, Creatinine 0.9 mg/dL. AST/ALT and Bilirubin within normal limits.
- Urinalysis: Clear, negative for nitrites, leukocyte esterase, or blood.
- Imaging: CT Abdomen and Pelvis with IV contrast is currently pending.

Assessment/Plan

1. **Right Lower Quadrant Abdominal Pain (R10.31):** High clinical suspicion for acute appendicitis given the classic migration of pain, RLQ tenderness, and mild leukocytosis. Nephrolithiasis and other GI etiologies are less likely but on the differential.

- **Plan:**
 - Keep patient NPO.
 - Placed peripheral IV. Administered 1L Normal Saline bolus.
 - Administered Ondansetron 4mg IV for nausea.
 - Administered Ketorolac 15mg IV for pain control.
 - Awaiting CT Abdomen/Pelvis with IV contrast.
 - General Surgery team has been notified and is on standby pending imaging results.

Provider Notes

Patient is hemodynamically stable at this time with an excellent blood pressure profile (120/80) and normal heart rate. Pain is currently well-controlled after initial IV medication. Will re-evaluate the patient as soon as the CT scan results are finalized to determine if surgical intervention is indicated or if he can be safely discharged with outpatient follow-up.